

Georg von Westphalen

GENERAL SURGERY: THE SURGICAL SCRIMMAGE

A chance to cut is a chance to cure. Stay fluffy!



Archie Appendicitis The Red Fuse

McBurney's Point
Alvarado Score
WBC ↑



POWER: The Pecalith Jam. Archie starts with a belly-button ache and ends with a sharp, right-sided stab.
LORE: Obstruction of the lumen leads to intraluminal pressure spikes, ischemia, and eventually a Blowout (Perforation).

KayfaBINGO!



Obstruction Otis The Bowel-Blocker

Dilated loops > 3cm
High-pitched tinkling



POWER: The Traffic Jam. Otis stops the flow. Everything upstream swells; everything downstream is The Ghost Town.
LORE: Most often caused by Adhesions (scar tissue). Otis leads to severe Vomitus and distension.

HATY
RAT-KING!



Peritonitis Pete The Toxic Flood

Rebound tenderness
Rigid Board-like abdomen



POWER: The Chemical Riot. Pete floods the peritoneal space with bile, stool, or blood, turning the ring into a toxic swamp.
LORE: Inflammation of the serous membrane causes a massive shift of fluid, leading to hypovolemic shock if not drained.

Volvulus Val The Twist

Coffee Bean Sign
Sigmoid vs. Cecal



POWER: The Strangler. Val twists the bowel around its own blood supply (Mesentery), cutting off oxygen instantly.
LORE: This is a vascular emergency. If Val isn't Untwisted, the segment turns gangrenous in hours.

CHOLE-CATHY THE STONE COLLECTOR

RUQ Pain / Murphy's Sign (+) / 4 F's

POWER: The Gallbladder Squeeze. Cathy tries to push a stone through a tiny pipe after a fatty meal.

LORE: Cholecystitis occurs when a stone blocks the Cystic Duct, causing the Gallbladder Wall to swell and inflame.



PANCREAS-PHIL THE SELF-EATER

Lipase ↑↑↑ / Cullen's / Grey Turner's

POWER: The Auto-Digest. Phil gets angry and releases his enzymes inside himself, melting his own tissue.

LORE: Often triggered by Gallstones or Alcohol. Phil can trigger a deadly Cytokine Tsunami (SIRS).



JAUNDICE JACK THE YELLOW GHOST

Bilirubin ↑ / Pale Stools / Dark Urine

POWER: The Bile Backup. Jack appears when the Common Bile Duct is blocked by a stone or Pancreas-monster.

LORE: Conjugated bilirubin can't reach the gut, so it leaks into the blood and out through the skin and eyes.

CHOLANGITIS-CHUCK THE REYNOLDS PENTAD

Fever, Jaundice, RUQ Pain + Shock + AMS

POWER: The Sepsis Storm. The ultimate biliary emergency. Infection is trapped under pressure in the bile ducts.

LORE: Pus under pressure (Charcot's Triad +) is a death sentence without urgent decompression (ERCP).



BLÖDSINN!

9 Inguinal-Izzy ♣ The Groin-Bulge

Indirect vs. Direct
(Resselbach's)

POWER: The Escape Artist.
Izzy finds a weak spot in
the abdominal wall and
shoves the bowel through it.

LORE: Indirect Izzy
follows the Spermatic
Cord; Direct Izzy just
punches straight through
the Resselbach Floor.



♣ 6

10 Strangulated-Stan ♦ The Ischemia Trap

Non-reducible / Tender
Purple skin

POWER: The Death Grip.
The hole is too tight. Blood
can't get in, and the
trapped tissue starts to die.

LORE: A Reducible
hernia is a nuisance; a
Strangulated Stan is a
surgical race against
the clock.



♦ 10

11 Femoral-Fiona ♥ The Low-Bulge

Below Inguinal Ligament.

POWER: The High-Risk Escape.
Fiona uses a tiny, tight
canal. She is highly prone to
rapid Strangulation.

LORE: The Femoral Canal is
rigid and narrow. Fiona
doesn't play; she traps the
bowel almost immediately.



♥ 11

12 Incisional-Ina ♠ The Weak Link

Occurs at old
surgical sites.

POWER: The Scar-Breaker.
Ina pushes through the
ghost of a previous fight
where the Fascia-Arma
never healed.

LORE: Failure of primary
healing. Ina is the exact
reason surgeons obsess
over Suture Tension.



♠ 12

What the
FLUFF?!

What the
FLUFF?!



K
♠

Triple-A-Arthur

Abdominal Aortic Aneurysm

Pulsatile mass
Diameter > 5.5cm



POWER: The Silent Bomb. Arthur waits. He grows slowly and painlessly until he suddenly Pops.

LORE: Weakening of the tunica media. If Arthur ruptures, the Scrimmage moves to the mærgue without instant OR access.

♥
K

Q
♥

Mesenteric-Mitch

Bowel Ischemia

Pain out of proportion
Lactic Acid ↑



POWER: The Gut Stroke. Mitch cuts off blood to the intestines. The patient screams, but the belly feels soft.

LORE: Embolism in the SMA. Without blood, the bowel lining dies, leading to massive bacterial invasion.

♥
Q

J
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Varicose-Vince

The Vein-Bloater

Lower limb heaviness
Trendelenburg test



POWER: The Backflow. Vince occurs when the valves in the legs fail, allowing venous blood to pool in the ankles.

LORE: Chronic venous hypertension leads to severe skin changes and Woody Induration (Lipodermatosclerosis).

♣
J

10
♦

Claudication-Claude

PAD

Ankle-Brachial Index (ABI) < 0.9



POWER: The Walking Stop. Claude starves the muscles of oxygen, making the calves burn after just 100 meters.

LORE: Atherosclerosis of the iliac/femoral arteries. Claude is a blaring warning siren of Global Vascular War.

♦
10



WayfaBONGO!

17 Peptic-Percy



The Ulcer

H. pylori (+) /
Coffee-ground emesis

POWER: The Acid Burn. Percy eats through the stomach lining. If he goes deep, he hits the Gastroduodenal Artery.

LORE: Imbalance between gastric acid and mucosal protection. Perforation leads to lethal Free Air under the diaphragm.



17

What the FLUFF?!



18 Mallory-Weiss Mel



The Tear

Hematemesis after
forceful retching

POWER: The Retch-Rip. Mel occurs when violent vomiting tears the delicate junction between esophagus and stomach.

LORE: A longitudinal mucosal tear. Mel usually stops bleeding on his own, but the hematemesis is terrifying.

18



19 Boerhaave-Bob

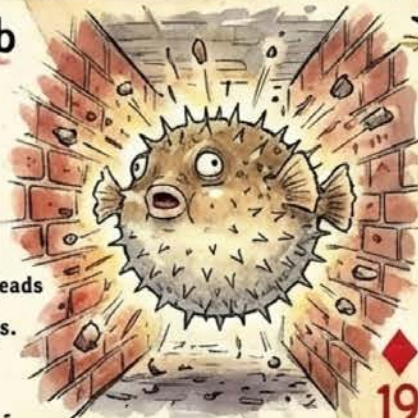


The Rupture

Mackler's Triad

POWER: The Full-Thickness Blowout. Bob is the Final Level of Mel. The esophagus actually pops entirely open.

LORE: Transmural rupture leads to Mediastinitis—bacteria flooding the heart and lungs. A 911 absolute emergency.



19

20



Achalasia-Ace

The Tight Throat

Bird's Beak
on Swallow

POWER: The Food Trap. Ace keeps the Bottom Door of the esophagus locked shut. Food sits, stacks up, and rots.

LORE: Loss of the Myenteric Plexus. Ace needs a surgical Heller Myotomy to forcefully kick the door down.

20



GW

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KayfaBONGO!

21 Divertic-Dan ♣ The Pouch-Pop

POWER: The Left-Sided Appendicitis. Dan occurs when tiny structural pouches in the colon get clogged and infected.

LORE: Diverticulosis is simply having the pouches; Diverticulitis is when Dan starts the inflammatory riot.



LLQ Pain/
CT scan
diagnosis

♣ 21

22

Colonoscopy
Screen

Poly-Pete The Polyp Screen

POWER: The Silent Grower. Pete isn't cancer yet, but he's Applying for the Job.

LORE: Villous adenomas carry the highest risk. Snipping Pete out during a colonoscopy stops the war before it begins.

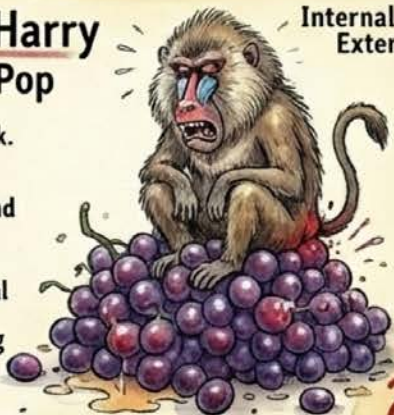


♦ 22

23 Hemorrhoid-Harry ♥ The Cushion-Pop

POWER: The Pressure Leak. Harry occurs when the venous cushions in the Basement (Anus) swell and prolapse out.

LORE: Heavy straining increases intra-abdominal pressure, forcing Harry violently out of his hiding spot.



Internal vs.
External

♦ 23

24



Fissure-Fred The Tear Sentinel pile

POWER: The Knife-Cut. Fred is a small tear in the anal lining that refuses to heal because the Sphincter-Muscle is spasming.

LORE: High sphincter tone causes localized ischemia. Nitro-cream or Botox can force the muscle to relax and let Fred heal.

♥ 24

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✕ **Thyroid-Theo**
♠ **The Butterfly**

TSH ↓
T4 ↑



POWER: The Metabolic Overdrive. Theo controls the match speed. If he goes hyper, the heart races and heat rises.
LORE: Cold nodules are more likely to be malignant. Theo's biggest surgery risk is severing the Recurrent Laryngeal Nerve.

♥ 25

♥ **Para-Thyroid Pat**
♥ **The Calcium Thief**

Calcium ↑
PTH ↑



POWER: The Bone-Breaker. Pat violently steals calcium from the bones and dumps it into the blood and kidneys.
LORE: Usually a single Adenoma (Pat's hideout). Surgery to evict Pat fixes the classic Stones, Bones, Groans, Moans.

♥ 26

27 **Adrenal-Addie**
♣ **The Stress Queen**

Pheochromocytoma
VMA



POWER: The Adrenaline Surge. Addie creates sudden, massive spikes in blood pressure that can blow a vascular fuse.
LORE: Rule of 10s: 10% bilateral, 10% malignant, 10% extra-adrenal. Always Alpha-block before you Beta-block!

♣ 27

28 **Insulin-Ian**
♦ **The Sugar-Dropper**

Whipple's Triad



POWER: The Crash. Ian releases too much insulin, causing massive hypoglycemia and making the patient faint in the ring.
LORE: An islet cell tumor of the pancreas. Ian is incredibly rare but acts as a highly dangerous metabolic assassin.

♦ 28

HUMBUG!

FAKE NEWS!

FAKE NEWS!

HE'S A HACK!

GW

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29 Scalpel-Stan
The Blade



#10, #11, #15

POWER: The Access Point. Stan achieves the very first goal of any surgical encounter: Exposure.

LORE: Stan must meticulously follow Langer's Lines to ensure the final scar heals like a whisper instead of a scream.



67

30 Bowie-Bill
The Electrosurgery



Cut vs. Coag



POWER: The Saoke & Sissle. Bill uses high-frequency sparks to cut tissue and literally Fry bleeding vessels shut.
LORE: Bill absolutely requires a Grounding Pad (Return Electrode) or the patient receives a severe accidental burn.

03

31 Laparo-Leo
The Minimalist



CO2 Insufflation



POWER: The Keyhole View. Leo uses a camera and long chopsticks to conduct the entire fight through a tiny hole.
LORE: Decreases pain and hospital time. Beware of the Phrenic-Phantom (rsferred shoulder pain from trapped CO₂ gas).

13

32 Suture-Sam
The Stitch



Absorbable vs. Permanent



POWER: The Structural Knot. Sam securely holds the Fascis-Armor together until the body can build its own bridges.
LORE: Tensile Strength is the absolute law. Sam is the sole reason the Belly-Burst (Dehiscence) doesn't happen.

32

KayfaBIZARRO!



33 Triage-Tim ♠ The Referee

Red, Yellow, Green, Black



POWER: The Life-Sorter. Tim acts as the gatekeeper, deciding who gets the Blade-Brigade first and who has to wait.
LORE: In a mass-casualty Scrimmage, you don't save everyone; you ruthlessly optimize to save the most people possible.



34 FAST-Fiona ♥ The Ultrasound

4 Windows



POWER: The Fluid-Finder. Fiona searches for hidden, pooling blood in Norison's Pouch or around the heart.
LORE: A positive Piona test means the patient is bleeding internally—they bypass the ER and go straight to the OR!



35 Chest-Tube-Chester ♦ The Drain

5th Intercostal Space



POWER: The Lung-Reinflator. Chester aggressively drains blood or air to let the crushed lungs breathe again.
LORE: The tube **MUST** stay above the rib margin to avoid lacerating the Neurovascular Bundle (The chest's wiring).



36 GCS-Gabe ♣ The Brain-Check

Total score 3-15



POWER: The Consciousness Meter. Gabe instantly tells the trauma surgeon exactly how hard the brain was hit.
LORE: GCS of 8, Intubate! If Gabe calculates the score is critically low, the patient's airway is no longer safe.



FAKE NEWS!



CHAOTIC FERAL CRITTERS

37 Malignant-Max Breast Cancer

BI-RADS score **M**



W

POWER: The Invasive Lump. Max hides in the upper-outer quadrant and slowly spreads his tendrils to the Sentinel Node.
LORE: Targeted therapy (Tamoxifen) or Wide Local Excision (Lumpectomy) act as Max's surgical kryptonite.

M Melanoma-Mel ★ The Mole-Ninja

ABCDE / Breslow



W

POWER: The Deep Dive. Mel starts superficially as a dark spot but rapidly dives deep to access the blood-highway.
LORE: Depth is everything. A 1mm depth is the clinical line in the Sand that mandates significantly wider surgical margins.

38 Sentinel-Sid The Node

Technetium-99/
Blue Dye **S**



POWER: The First Sentry. Sid is the very first lymph node that Max tries to infect. If Sid is clear, the army is safe.
LORE: Performing a Sentinel Node Biopsy strictly prevents the permanent Lymphedema-Leak by sparing unnecessary nodes.

Burn-Ben The Rule of 9s

1st to 3rd Degree **B**



B

POWER: The Fluid-Leech. Ben destroys the skin-barrier, letting all the body's water evaporate and bacteria instantly invade.
LORE: The Parkland Formula is Ben's specific counter-spell. Massive volume IV resuscitation is the only key to survival.



41 Dehiscence-Dan The Burst

5-8 days
post-op



POWER: The Wound-Split. Dan occurs when Suture-Sam's knot fails and the carefully constructed Belly-Armor tears open.

LORE: If the deep fascia fails and the guts physically fall out, it escalates to Evisceration-Eve—an absolute surgical nightmare.



17

43 DVT-Doris The Clot

Virchow's
Triad



POWER: The Leg-Jam. Doris stops the blood in the calf. If she dislodges and moves to the lungs, she becomes the deadly PE-Phantom.

LORE: Prophylactic Lovenox injections and early walking/ambulation are Doris's sworn enemies.



17

42 Atelectasis-Al The Lung-Collapse

Day 1-2
post-op fever



POWER: The Shallow Breath. Al closes the tiny Alveolar Balloons because the patient is in too much pain to breathe deeply.

LORE: The Incentive Spirometer is the physical counter-spell. You have to force Al to expand and breathe!



17

44 Post-Op-Paul The Ileus

Absent bowel
sounds



POWER: The Sleepy Gut. Paul happens when the bowels are handled too roughly during surgery and decide to Go on strike.

LORE: Paul is usually temporary. Surprisingly, Chewing Gum and early walking trick the nervous system into waking him up.



17





Fever-Fiona

The 5 Ws



POWER: The Temperature Spike. Fiona tells the surgeon exactly where the hidden infection is located based on post-op timing.
LORE: Day 1 = Wind (Lungs). Day 3 = Water (UTI). Day 5 = Wound. Day 7 = Walking (DVT).

Sepsis-Sam

qSOFA ≥ 2

POWER: The Total-Body Riot. Sam turns a minor local wound into a global vascular collapse. BP bottoms out; organs fail.

LORE: Massive vasodilation. Sepsis-Sam desperately needs 30ml/kg of Crystalloid Juice and immediate broad-spectrum IV antibiotics.



Hemorrhage-Hank

HR $\uparrow$ / BP $\downarrow$

POWER: The Tank-Drain. Hank is the silent killer. The blood is actively leaving the system, and the patient is getting dangerously sleepy.

LORE: Class I to IV hemorrhagic shock. By Class III, crystalloids aren't enough—you need Liquid Gold (Blood transfusions).



Anesthesia-Annie

End-tidal CO₂ $\uparrow$

POWER: The Metabolic Inferno. A rare, lethal reaction to anesthetic gas that turns the patient's body into an uncontrollable oven.

LORE: Defect in the Ryanodine receptor. The specialized drug Dantrolene-Dan is the absolute only weapon that can stop Annie.



49 Consent-Clerk Ethics

Risks, Benefits, Alternatives

POWER: The Legal Shield.
Ensures the patient actively
knows exactly what Anatomical
Trespassing is about to legally
occur.

LORE: Emergency-implied
consent only exists when
the patient is completely
unconscious, dying,
dying, and unable to speak
for themselves.



50 Scrub-Siren Sterility

3-minute scrub

POWER: The Germ-Wall.
The Siren rigidly ensures
no Microbe-Monsters hitch
a ride into the sterile
Surgical Scrimmage.

LORE: Blue is Sterile. In
the operating theater,
anything that isn't draped
in sterile blue is treated
as a lethal threat.



51 Pathology-Pack The Biopsy

Formalin fix /
Frozen section

POWER: The Final Verdict.
They dictate via microscope
if the Cancer-Monster is
fully gone, or if the surgeon
needs to cut wider
margins.

LORE: Frozen sections give
an intraoperative answer in
minutes; permanent
Formalin sections take days
to process.



52 Recovery-Ranger PACU

Aldrete Score

POWER: The Wake-Up Call.
The Ranger guards and
monitors the patient as
their brain slowly reboots
from deep chemical
paralysis.

LORE: Airway, Pain, and
Puke are the three deadly
enemies that the Ranger must
fight in the Post-Anesthesia
Care Unit.



53

The Boss The Attending

Board Certified

POWER: The Master-Trespasser. He oversees the Scrimmage, ensuring no one blindly cuts without understanding the Lore.

LORE: Is the patient stable? Is the anatomy clear? Why are you still holding the Bovie when you should be tying? Stay fluffy!



54

Quest 1: Murphy Trap Synthesis Scenario

Scan vs. OR

POWER: Scenario: A 45yo female presents with sharp RUQ pain after eating a pizza. You press under her ribs, and she stops breathing.

LORE: Identify the Stone-Collector (Chole-Cathy) causing this acute cholecystitis. Is this a conservative scan day or an urgent OR day?



55

Quest 2: Silent Rupture Synthesis Scenario

Evisceration Risk

POWER: Scenario: A patient 5 days after a complex colon resection reports a sudden Pop and serosanguinous fluid leaking from the wound.

LORE: Identify the Burst (Dehiscence-Dan). Assess the immediate mechanical risk of the Bowel-Spill (Evisceration).



56

Quest 3: Coffee-Bean Crisis Synthesis Scenario

Vascular Choke-point

POWER: Scenario: An elderly patient presents with a massive, hard belly. X-ray shows a giant Coffee Bean taking up the entire midsection.

LORE: Name the Strangler (Volvulus-Val) and urgently describe the exact mechanism of this life-threatening vascular choke-point.



Stay fluffy!

GW

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